SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Nintedanib + docetaxel

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Adenocarcinoma NSCLC · combination phase · NCCP 00372 · version 6 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
To help control adenocarcinoma lung cancer after first-line chemotherapy; treatment benefit and side effects are reviewed together.

Schedule
Day 1: docetaxel drip, with no nintedanib. Days 2–21: nintedanib capsules twice daily. Repeat every 21 days for up to 6 cycles (about 18 weeks without delays); treatment may stop earlier. A later tablet-only phase needs a new team-confirmed plan.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Docetaxel
TAXANE CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Allergic reactions and fluid retention
- ☐ Hair loss, fatigue and mouth soreness
- ☐ Nerve symptoms that can persist
- ☐ Severe bowel inflammation and infusion-site injury

Nintedanib
ORAL ANGIOKINASE INHIBITOR

- ☐ Diarrhoea, nausea and vomiting
- ☐ Liver injury
- ☐ Bleeding, blood clots and high blood pressure
- ☐ Kidney injury or protein in urine
- ☐ Rare bowel perforation, ischaemic colitis or PRES

Discuss the goal, alternatives, important risks for you, fertility/pregnancy prevention, practical support and what would prompt changing or stopping treatment.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00372 v6.

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